

ANNUAL REPORT



Abbreviations3

Message from the Board Chair4

Country Director’s Note5

About ICRHK6

FY 2025 Highlights7

Thematic programmatic impact

- HIV/AIDS & STIs9-12
- MNCH13-15
- Gender & GBV16-20
- Child protection21-23
- Adolescents & Youth Health24 -26
- Monitoring, Evaluation & Learning27

Innovation Hub28-29

Snapshots Corner.....30-32

Our Team.....33

Our Partners34

Our Publications35-37

Abbreviations

AGYW	Adolescent Girls and Young Women
AIGA	Alternative Income Generating Activities
ANC	Antenatal Care
CoG	Council of Governors
CSOs	Civil Society Organizations
DICES	Drop-In Centres
EWENE	Every Woman, Every Newborn Everywhere
FSW	Female Sex Worker
FTYM	First Time Young Mother
GBVRC	Gender-Based Violence Recovery Centre
HIV	Human Immunodeficiency Virus
HMB	Heavy Menstrual Bleeding
ICRHK	International Centre for Reproductive Health-Kenya
KDHS	Kenya Demographic and Health Survey
KNBS	Kenya National Bureau of Statistics
KP	Key Populations
KPLHIV	Key population living with HIV
MHHM	Menstrual Health and Hygiene Management
MNCH	Maternal, Newborn and Child Health
MSM	Men who have Sex with Men
NASCOP	National Aids and STI Control Program
NGO	Non -Governmental Organization
PLHIV	People Living With HIV
PMA	Performance Monitoring Accountability
PrEP	Pre-exposure prophylaxis
PSK	Population Services Kenya
SRH	Sexual and Reproductive Health
SRHR	Sexual and Reproductive Health Rights
VL	Viral Load
SNS	Social Network Strategies
TWG	Technical Working Group

Message from the Board

Chairperson



Professor Kishor Mandaliya

Board Chairperson

The year 2025 marked a significant milestone for International Centre for Reproductive Health Kenya (ICRHK) as we celebrated 25 years of advancing Sexual and Reproductive Health and Rights (SRHR). This anniversary came at a time of profound shifts in global development financing-changes that tested the resilience of many institutions while reaffirming ICRHK's enduring relevance within Kenya's health ecosystem.

Across the sector, organizations faced what has been described as a "funding cliff," driven by the gradual withdrawal of long-standing development assistance, including traditional USAID funding streams. For ICRHK, however, this moment also demonstrated the institutional maturity and strength built over the past quarter century.

In response, the Board of Directors remained steadfast in its commitment to safeguarding the organization's mission. Our responsibility to the communities we serve- particularly key populations who continue to face significant barriers to health and protection- extends far beyond the life-cycle of individual grants.

When external funding for our Drop-In Centers (DICEs) was threatened, the Board supported a strategic shift toward the use of internal resources to sustain these essential services. This decision

ensured continuity of life-saving care and reflected ICRHK's integrity, fiscal discipline, and commitment to the communities that have relied on our support for over two decades.

As we conclude the 2020–2025 Strategic Plan, the Board is focused on positioning ICRHK for the future. Our next phase will emphasize diversified financing, strengthened partnerships, and innovative approaches to address emerging health challenges.

On behalf of the Board of Directors, I extend sincere appreciation to our staff, partners, donors, and the Government of Kenya for their continued trust and collaboration. With strong foundations built over the past 25 years, ICRHK is well positioned to expand its impact and continue advancing health, dignity, and equity across Kenya and the region.

Country Director's Note

For ICRHK, 2025 was a year of transformation and progress. In an environment defined by shifting global health priorities and changing funding patterns, the organization demonstrated that the global call for localization is not simply an aspiration- it is a reality we are actively shaping.

During the year, ICRHK successfully transitioned from primarily serving as a sub-grantee to managing direct grants as a Prime Recipient. This shift strengthened our institutional independence and enabled us to expand our presence into new counties across Kenya, demonstrating the strength, credibility, and scalability of our operational systems.

Despite a challenging financing environment, our technical portfolio continued to evolve. New specialized grants allowed us to address emerging and often overlooked women's health priorities, including Heavy Menstrual Bleeding (HMB), while maintaining strong programming across our established areas of work.

Our teams continued to deliver meaningful results for communities across the country. Through the Gender-Based Violence Recovery Centre (GBVRC), survivors received comprehensive medical, psychosocial, and legal support. Through the Okoa Mama na Mtoto Initiative, ICRHK contributed to the development of the national Every Woman, Every Newborn Everywhere strategy, helping elevate maternal and newborn health priorities within national policy discussions. At the same time, our HIV programs provided life-saving services to more than 16,600 members of key populations.

These achievements were made possible through strong and enduring partnerships. We worked closely with government institutions, academic and research partners, and civil society organizations across global, national, and community levels to ensure that our interventions remain evidence-informed, responsive to local needs, and aligned with national health priorities.

The dedication and professionalism of our staff remain the driving force behind ICRHK's impact.



Dr Susan Ontiri

Country Director/CEO

Even in the face of operational uncertainty, our teams sustained high-quality programming, generated valuable evidence, and contributed to policies that strengthen health and equity.

I thank all our stakeholders for their continued trust and collaboration. Together, we move into the next quarter-century committed to expanding opportunity, strengthening health systems, and improving the lives of communities across Kenya and beyond.

About ICRHK

International Centre for Reproductive Health- Kenya (ICRHK) is a local women-led non-governmental organization established in 2000 and dedicated to advancing sexual and reproductive health and rights (SRHR) through research, advocacy, and interventions.

Over the last 25+ years, since ICRHK was officially registered, we have expanded our footprint to drive evidence-based interventions in 33 counties across Kenya, bringing life-changing SRHR research and programs to communities.

VISION

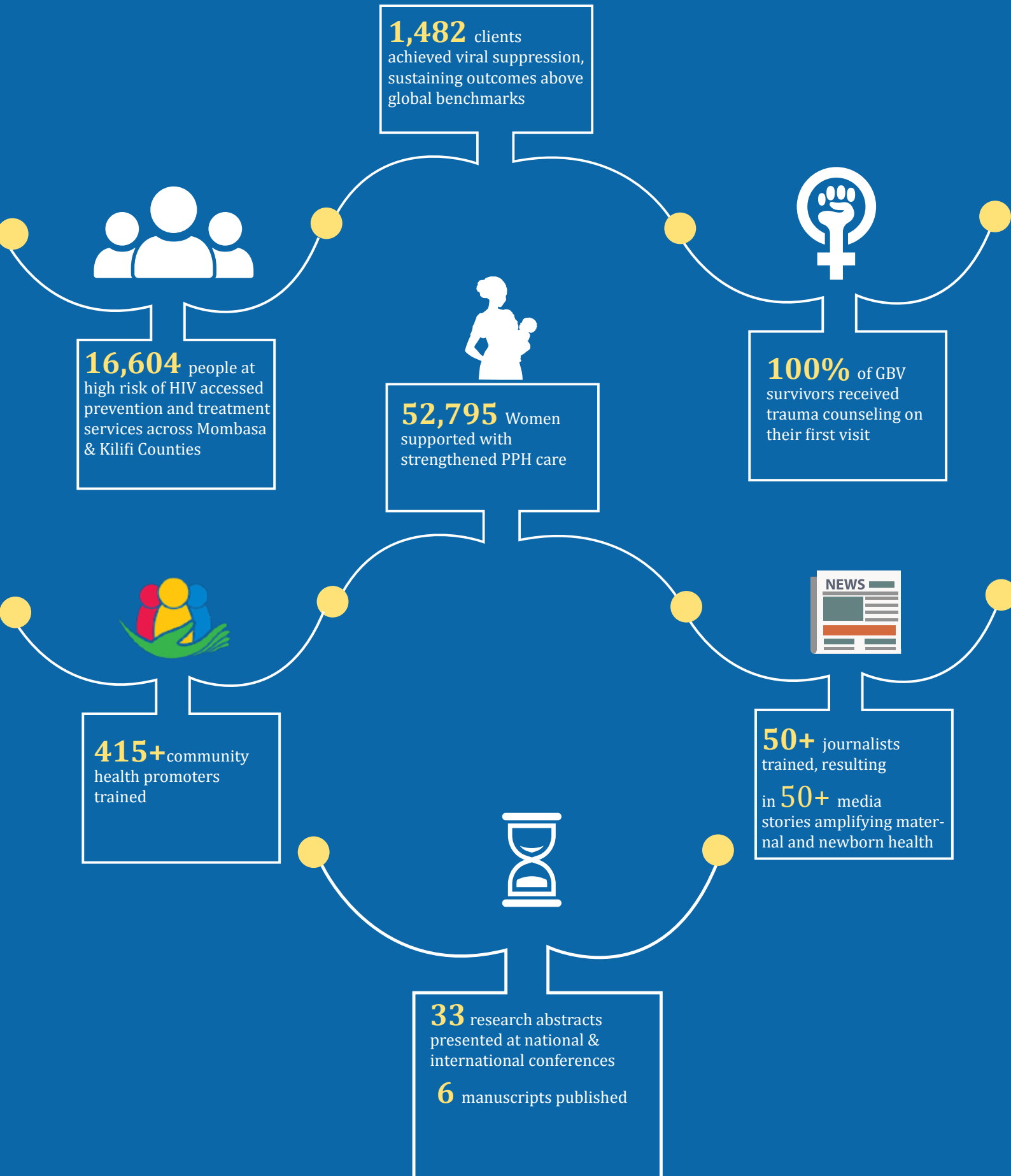
To be a Centre of excellence in the implementation of research and interventions in public health, including sexual reproductive health



MISSION

To contribute to improved health status of communities by conducting high quality research, designing and implementing innovative, evidence-based and cost-effective interventions capable of influencing public policy and practice and capacity building in the field of public health including sexual and reproductive health in Kenya.

FY 2025 Highlights





HAVE YOU BEEN RAPED?

Report to the nearest hospital within 72hrs!

DO'S

- 1. Report to the nearest hospital immediately for:
 - HIV Prevention (PEP)
 - Pregnancy prevention
 - Treatment of Sexually Transmitted Infections
 - Collection of evidence
 - Counseling
- 2. Report to the police and record a statement.

DO NOT'S

- 1. **DO NOT** take a bath or wash your hair.
- 2. **DO NOT** change, wash or throw away your clothes. (Wash them in a black bag, keep them in nylon cloth and take them to the hospital).
- 3. **DO NOT** change anything where the rape occurred.
- 4. **DO NOT** dispose off a condom if one was used.
- 5. **DO NOT** come back to the area where the incident took place. If you have to, take a taxi, collect your belongings and take it to the hospital.

HIV, Sexually Transmitted Infections can be prevented if you report to a hospital within 72 hours of the rape/sexual assault.

REMEMBER: IT IS NOT YOUR FAULT THAT YOU WERE RAPED. DO NOT FEEL GUILTY, DO NOT BE ASHAMED, DO NOT TAKE ACTION AND REPORT TO THE POLICE.

Call 1190 for free professional counselling & support.

Client Flow Pathway for Treatment of Survivors of Sexual Violence

Survivor should be attended to as priority



Millicent Akinyi, Community health coordinator providing service to a client at Kisauni DICE

HIV/AIDS & STIs



A client getting tested during Nuru ya Vijana Outreach activity

In FY25, ICRHK strengthened community-led HIV prevention and treatment systems, ensuring Most At-Risk Populations could access care safely and consistently through programs including Stawisha Pwani, Nuru ya Vijana, MSF project, and the TIKO project. For many key populations in coastal Kenya, accessing HIV services still means navigating stigma, insecurity, and structural exclusion.

Across Mombasa and Kilifi Counties, **16,604** key populations accessed services, achieving 99% of the annual target. Through differentiated service delivery anchored in Drop-In Centers (DICES), clients received targeted HIV testing, same-day ART initiation, viral load monitoring, PrEP and PEP, STI treatment, mental health support, family planning services and SGBV response services.

Despite periodic sociopolitical disruptions and testing commodity constraints, treatment outcomes remained strong. Viral suppression was sustained above global benchmarks, reinforcing Kenya's progress toward ending HIV as a public health threat by 2030 and demonstrating the effectiveness of community-centered models in complex environments.

IMPACT

16,604	Key populations who accessed services
15,616	Key populations tested for HIV
155	Key populations newly diagnosed with HIV
1,549	Current on ART
1,997	Clients eligible for viral load testing
1,501	Received viral load results
1,482	Achieved viral suppression
100%	Linked to care
100%	Initiated on treatment
3,963	Clients enrolled on PrEP

Social Network Strategies (SNS)

SNS enabled key populations, both HIV positive and high-risk negative, to extend HIV testing and SRH services through their social networks. The strategy provided HIV testing & linkage, PrEP, and other SRH services, supported by trained clinicians implementing the strategy.

IMPACT

303

Key populations served as “seeds” to reach their social networks.

1,045

Coupons distributed & 899 returned, connecting 899 social contacts to HIV testing services.

87

Positive social contacts linked to care.

Project Highlights

Nuru ya Vijana

In FY25, Nuru ya Vijana advanced HIV and ASRHR access while strengthening economic resilience for young female sex workers and transgender persons through HIV testing, prevention and adherence, PrEP, skill mentorship, Alternative Income Generation, and capacity building through healthcare workers training, peer educators training and Community Advisory Board (CAB) trainings.

IMPACT

268

YKPs trained in financial literacy and business skills

46

Healthcare providers trained on YKP guidelines

Health services were also integrated into a community PRIDE event, providing HIV testing, health talks, and linkage to care.



Rose Gwada -Nuru ya Vijana Community health Coordinator showcasing the projects progress during a donor visit to the DICE

Genesis

Under the Evidence Generation for Sexually Transmitted Infection Point of Care Diagnostics in the Public Sector in Kenya (Genesis) project, whose goal is to generate lessons for scaling-up point-of-care (POC) diagnostics to improve sexually transmitted infections management and outcomes for low- and middle-income countries, in FY25, it built on the foundational work completed in Year 2024.

In FY25, the project focused on facility readiness, laboratory validation, field evaluation, and costing, in collaboration with national and county health teams.

IMPACT

41

Public health facilities (Nairobi & Kilifi) assessed for POC STI readiness.

Laboratory validation completed, including training staff, developing SOPs, and prepotency testing at KEMRI-CMR, Kisumu.

9

Abstracts presented at National and International Conferences



Nairobi County health management team and Genesis project team

Case Story:



Nuru ya Vijana peers learning soap making as an Alternative Income Generating Activity (AIGA)

In the heart of Kilifi County, Jane* joined the Alternative Income Generating Activity (AIGA) training under the Nuru ya Vijana project, seeking more than a skill; she was looking for a lifeline. Like many young peers in coastal Kenya, financial insecurity shaped daily choices and limited opportunities.

Through the Nuru ya Vijana project, young female sex workers and transgender persons were equipped with livelihood skills and financial literacy to build sustainable futures. Over three training cohorts in 2025, 90 young people learned to make detergents such as JIK, soap, fabric softener, and disinfectant products in high demand in their communities.

For Jane*, the training was transformative. “We were taught about self-management, knowing your worth, and having the confidence to start our own business,” she recalls. “We also learned entrepreneurship and financial management, how to conceptualize and manage a project, market products, and prepare a business to grow. I now understand how to diversify my business and make it better.”

Mary, an eight-month pregnant mother and fellow peer, was referred to the training by a peer educa-

tor. “I hope the skills I’ve learned will improve my finances,” she says. “This is not just about making products; it’s about building a future for myself and my child.”

Trainer Isaa Mongi, a certified ILO ToT, explains, “We start with self-management and self-awareness, teaching young people to value themselves and stand tall. Then we teach core business skills, entrepreneurship, financial management, and marketing strategies. Practical sessions, like making detergents and shampoos, give them quick wins they can turn into real income.”

The AIGA initiative goes beyond skill acquisition; it fosters confidence, independence, and resilience. By integrating skills training with financial education, young peers reduce economic vulnerability, build sustainable livelihoods, and positively impact their households and communities.

For Jane, Mary, and their peers, this program is more than training; it is the start of a journey toward self-reliance and empowerment. “This skill is with me for life,” Jane says. “It has given me hope and the belief that I can stand on my own.”



Maternal, Newborn and Child Health (MNCH)



In FY25, ICRHK advanced system-level solutions to reduce maternal deaths, strengthen maternal and newborn care, and institutionalize sustainable health practices. Through high-impact clinical interventions, structured mentorship, commodity alignment, and community accountability systems, the program supported women, families, health workers, and counties to deliver safer births and healthier newborns.

The most significant achievement was the demonstration of facility-level reductions in maternal mortality, with multiple referral hospitals sustaining zero maternal deaths over several months. These results validate the effectiveness of standardized PPH bundles, reliable commodities, and mentorship models.

Challenges faced included intermittent TXA stock-outs, procurement delays, infrastructure constraints in lower-level facilities, and variability in mentorship support. Despite these barriers, the MNCH portfolio maintained strong service delivery and laid the foundation for institutionalized, life-saving maternal care systems.

IMPACT

52,795

Women supported with strengthened PPH care

49,190

Women accessed life saving Heat Stable Carbetocin (HSC/Tranexamic Acid

693

Healthworkers trained

200+

National champions mobilized to advance maternal survival

Project Highlights

Okoa Mama na Mtoto Initiative (OMMI)

The OMMI consortium improved maternal and newborn survival in Kenya in 2025 by strengthening health systems, expanding domestic financing, and institutionalizing high-impact MNCH interventions. The project worked across counties and nationally to ensure safer births and healthier newborns through clinical mentorship, policy integration, financing advocacy, and community accountability.

NLUM-PPH COF Project (HSC & TXA Scale-Up)

In FY25, the NLUM-PPH COF project accelerated the introduction and scale-up of Heat-Stable Carbetocin (HSC) and Tranexamic Acid (TXA) as part of a standardized first-response bundle to prevent and treat postpartum hemorrhage (PPH). The project strengthened national coordination, expanded health workforce capacity, and improved service delivery across counties.

IMPACT

450+

MNCH champions mobilized to advance maternal survival

National EWENE Acceleration Plan developed

Kilifi RMNCAH Bill signed into law

415

Community Health Promoters trained

50+

Journalists trained on MNCH reporting

IMPACT

52,795

Women rsupported with strengthened PPH care

49,190

Women accessed life saving Heat Stable Carbetocin (HSC/Tranexamic Acid

Simulation-based learning with 50 Mama-Natalie bleeding simulators Implemented

Facilities supported in Kiambu, Kilifi, and Webuye achieved periods of zero maternal deaths

Case Story:



H.E. Priscilla Ambetsa crowning the First Lady of Kakamega County Professor Janet Barasa as the OMMI Champion for Maternal and Newborn Health.

In Kakamega County, H.E. Priscilla Ambetsa, Director of Touch a Life, and OMMI partner officially crowned the First Lady of Kakamega County as the OMMI Champion for Maternal and Child Health Survival. The ceremony, held at the First Lady's office, was more than symbolic; it marked a bold step toward mobilizing political goodwill and grassroots support for maternal, newborn, and child health (MNCH).

Taking on the role with determination, the First Lady pledged her full support to Barasa Care, Kakamega's maternal and child health program,

promising that expectant mothers would walk through pregnancy safely, deliver safely, and return home with healthy babies. She also encouraged women to register under the Social Health Authority (SHA), ensuring maternity care is affordable and accessible.

Through the combined efforts of county leadership, grassroots mobilizers, community health volunteers, and champions like Priscilla Ambetsa, the OMMI project continues to transform maternal and newborn health in Kakamega, ensuring no mother or child is left behind.

Thanks to OMMI's advocacy and community mobilization:

SHA registrations increased from 654,061 (24 May 2025) to 763,484 (15 July 2025), a surge of 109,423 people in under two months.

Maternal deaths dropped from 14 in Q3 to 7 in Q4, a 50% reduction.

Funding commitments were secured, including KES 2 million for RMNCAH and KES 1 million for Family Planning, with advocacy ongoing to expand support



ICRHK team marking the 16 Days of Activism 2025

Gender and GBV

In FY25, ICRHK strengthened its 18-year legacy in GBV prevention and response through an integrated approach combining survivor-centered care, rigorous research, and policy advocacy. For many women, girls, and vulnerable populations in Kenya, accessing support for gender-based violence (GBV) often involves navigating stigma, limited services, and social or legal barriers.

By supporting **10,608** survivors at the Gender-Based Violence Recovery Centre (GBVRC) since its inception in 2007 and generating critical data through the PMA Agile and What Works projects, in FY25, ICRHK continued to drive measurable progress toward Sustainable Development Goal 5.

Through sustained programming, ICRHK demonstrated that integrating medical response with legal and psychosocial support underpinned by data yields the most effective outcomes for women, girls, and vulnerable populations.



A play therapy counselling session ongoing at the GBVRC Centre



Mentorship session for teen mothers at Mikindani led by the GBVRC team

IMPACT

GBVRC Service Delivery (FY25)

373

Survivors accessed services including medical, psychosocial and legal support

100%

Survivors received trauma counselling on first visit

48%

Accessed PEP within 72 hours

IMPACT

Research & Evidence Generation

PMA Agile Study: Data on GBV and SRH among youth validated by KNBS

What Works RCT: 800 couples enrolled in the What Works RCT to evaluate IPV reduction and gender equity improvements

Project Highlights

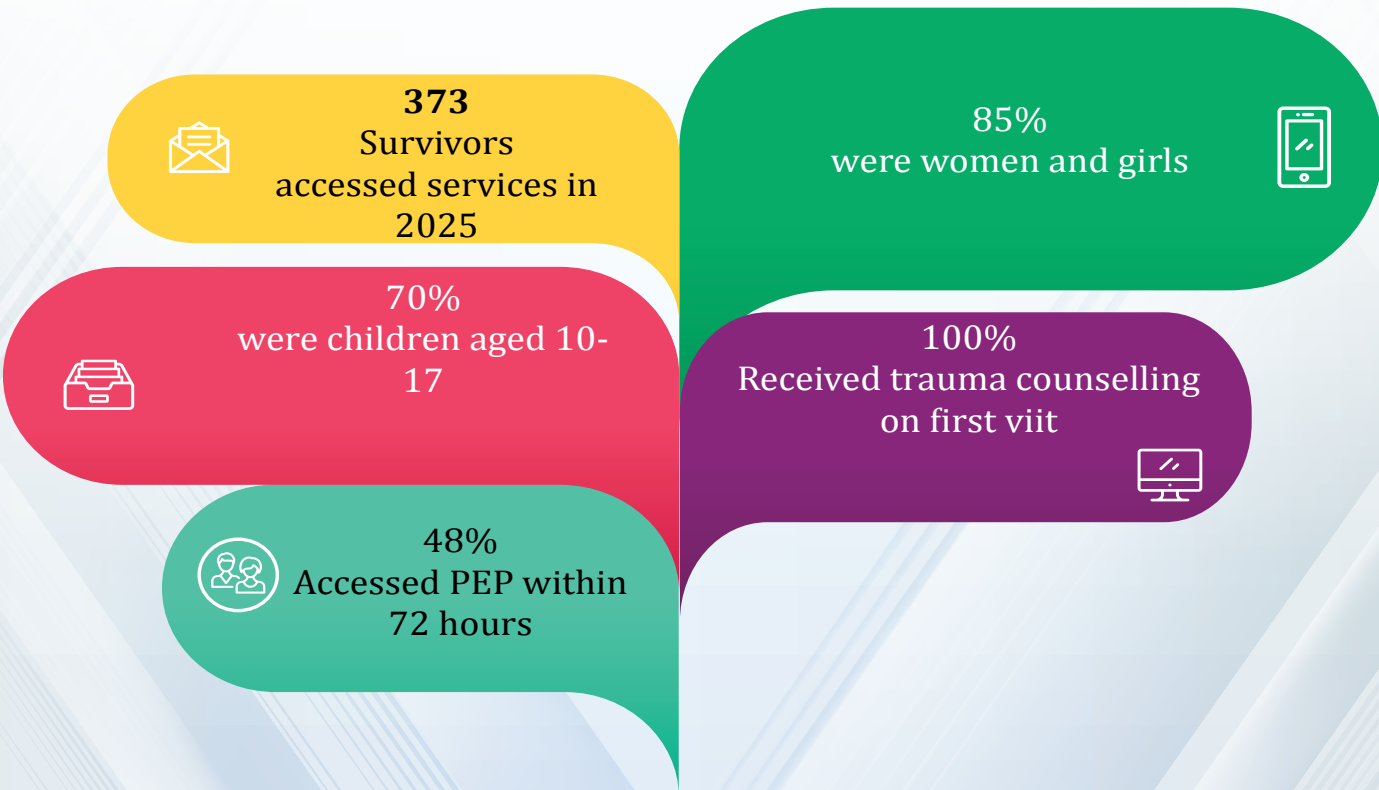
Gender Based Violence Recovery Centre (GBVRC)

The Gender-Based Violence Recovery Centre (GBVRC) at Coast General Teaching and Referral Hospital continued to operate as a model “One-Stop Centre” in FY25. The Centre prioritized restoring dignity and ensuring justice through its comprehensive service delivery model.

Why GBVRC Excels

- Integrated Care: Medical, psychosocial, and legal services under one roof
- Child-Focused: Pediatric trauma care and specialized therapy rooms
- Community-Connected: Strong networks with paralegals and police gender desks
- Sustainable: Embedded in a public referral hospital for long-term impact

FY2025 Service Delivery Highlights



Research and Evidence Generation

PMA Agile Project

In FY25, the PMA Agile Project strengthened evidence on gender-based violence (GBV) and sexual and reproductive health (SRH) among youth in Nairobi, supporting data-driven decision-making and policy advocacy. In collaboration with Kenyatta University, JHU, KNBS, and NCPD, the project ensured youth-focused research directly informed community and national action.



What Works Project

Partnering with Johns Hopkins University (JHU) and the Global Women's Institute, What Works Initiative was launched where ICRHK is evaluating the CREAM Kenya "Imarisha Mahusiano" couples-based program in Meru and Tharaka Nithi counties. Through this prevention research project, in FY25, ICRHK advanced evidence on preventing violence against women and girls,, generating actionable evidence for gender-transformative interventions.

IMPACT

- PMA Agile Data validated by KNBS as "fit for purpose" for official national reporting
- 200+ community stakeholders reached through dissemination and engagement
- 10,000+ social media impressions generated, amplifying evidence for dialogue and advocacy

MILESTONES

- 800 couples enrolled in a Randomized Controlled Trial (RCT)
- Baseline data collected
- Provides evidence to scale prevention strategies nationally and contribute to the global knowledge base

PMA Agile team led by the Project Director MARY tHIONGO during the dissemination of findings of PMA Agile 2.0 Nairobi Youth Cohort 2024 Survey



Healing Through Talking

In FY25, Fund Marleen Temmerman granted ICRHK an award to enhance its Gender Based Violence work at the Gender Based Violence Recovery Centre (GBVRC). This gave birth to Healing Through Talking study, an 18-month research initiative, aimed at strengthening psychosocial support for survivors of gender-based violence at the GBVRC.

The study evaluates the effectiveness of a structured psychosocial support (PSS) counselling intervention in improving attendance, adherence, and completion of counselling sessions.

It compares three approaches: standard GBV psychosocial care, group therapy, and hybrid/tele-counselling, to identify scalable, survivor-centered models that respond to barriers such as stigma, time constraints, and access to services.



A counselling session ongoing at the Gender Based Violence Recovery Centre (GBVRC)



A debrief session among the healthcare providers at the Gender Based Violence Recovery Centre (GBVRC)

STRIDES for SRHR through Resilient Health Systems

ICRHK received a new grant and working collaboratively with partners launched the STRIDES project in Kilifi County. This is a seven-year initiative funded by the Government of Canada and implemented with Aga Khan University (AKU), Aga Khan Health Services (AKHS), International Centre for Reproductive Health -Kenya (ICRHK), Moving The Goalposts (MTG), and the Kilifi County Government.

The project aims to strengthen sexual and reproductive health and rights (SRHR), reduce teenage pregnancy and gender-based violence (GBV), and improve linkages between communities and health systems. ICRHK leads clinical and facility-based interventions, supporting the delivery of integrated, youth-friendly, and inclusive services.



ICRHK Team together with other partners at the launch of Strides project in Kilifi.

Guardian of a Linda Mtoto beneficiary who was trained on Alternative Income-Generating Activities and, as a result, started farming. Her farm now feeds her family and provides income to educate her children.



Child Protection



ICRHK continued to strengthen child protection systems in Mombasa County, focusing on the prevention of sexual abuse and survivor-centered responses for children at heightened risk, including those affected by Commercial Sexual Exploitation of Children (CSEC).

Through the Linda Mtoto project, our interventions reached Jomvu and Kisauni sub-counties, combining mentorship, psychosocial support, child protection education, and community engagement to safeguard children and strengthen resilience.

Across our child protection programs, **885** children were identified, with **400** children aged 12–17 actively engaged in mentorship and psychosocial support. Child-led support groups provided safe spaces for disclosure, peer learning, and life skills development, complemented by referrals to critical services based on individual needs.

These interventions contributed to reducing violence and exploitation, supporting school retention, and building long-term resilience, while reinforcing community and system capacity to prevent, identify, and respond to abuse.

IMPACT

169	Children supported with education packages
400	Children Mentored
400+	Caregivers Trained
11	Caregiver-led self-help groups established

Case Story

When an Intervention Becomes a Lifeline

She sits quietly on a rock in the middle of the compound, watching the sun sink into the horizon. The day is almost over, but her thoughts are alive with the memories of her journey, from the challenges she has faced to the hope she still holds close to her heart. Earlier today, she attended a Linda Mtoto peer session, where participants reflected on what the project has achieved and what the future may hold as it transitions. For Lily*, this reflection is deeply personal.

"I live in Bangladesh," she begins. "I joined the Linda Mtoto project when I was in class seven. Our Community Health Promoter came to school one day, called us out of class, and told us about an ICRHK meeting. Because she was a respected woman from our neighborhood, my mother allowed me to go, and that's how my journey with ICRHK started."

Linda Mtoto, a project under International Centre for Reproductive Health- Kenya (ICRHK), works to protect children from abuse, neglect, and exploitation while ensuring that they remain in school and thrive.

Lily's path to education was not smooth. In 2021, she sat for KCPE and scored 286 marks. But her father refused to take her to secondary school. He insisted she should go upcountry to live with her stepmother, and when she resisted, he withdrew all support. "It was like, if I wanted to study, then I had to go to the village," she recalls. "I ended up staying home for nine months."

The stigma of being out of school weighed heavily. In her community, children who are not in school are often looked down upon, and families are judged harshly. At one point, the police arrested children who were out of school, and Lily* was taken to the police station. She was later given a calling letter to a certain school within the neighborhood, but, again, without fees, her future looked uncertain. Still, she encouraged her mother not to lose hope.

During one Linda Mtoto meeting, she gathered the courage to speak directly to the project officer. She asked if they could help her pay school fees. Her plea was heard. In February 2022, she received a cheque from Linda Mtoto, which allowed her to go back to school and pay her fee.

Her mother, who sells food near a godown, could barely manage rent, let alone school fees. "My father didn't believe I had gone back to school. He thought without his money, I could not study," Lily* says. "My parents are separated, so I depended only on my mother."

What stands out in Lily's story is her resilience. "I thank God be-



cause I managed to return to school when many of my peers didn't," she says. "Some gave in to peer pressure, got married, or even had children. But I was able to go back."

Through Linda Mtoto, Lily found not only education but also dignity. "Before, I was seen as someone who couldn't make it, as if our family was nothing. But ICRHK, through Linda Mtoto gave me back my self-esteem.

Adolescents & Youth Health

During FY25, our efforts focussed on strengthening adolescent and youth sexual and reproductive health (AYSRH) systems by advancing national policy development, legislative engagement, and evidence-informed advocacy.

Our work centered on engaging policymakers and technical actors to integrate adolescent-responsive priorities into policies and guidelines, while ensuring the needs of girls, marginalized adolescents, and young people with disabilities were reflected in planning and implementation processes.

Collectively, these efforts contributed to more evidence-informed, inclusive, and responsive adolescent health policies, dialogue and interventions.

ICRHK also served as a core member of the Ministry of Health task force that developed

Kenya's first Family Planning Policy, contributing to stronger adolescent inclusion.

Through engagements with parliamentarians, the National Council for Persons with Disabilities, and the ASRH Technical Working Group, the program elevated menstrual health, disability inclusion, teenage pregnancy prevention, and protection against SGBV within national dialogue.

These efforts reinforced Kenya's response to rising teenage pregnancy and adolescent vulnerability, strengthened evidence-driven decision-making, and advanced progress toward SDG 3 (Good Health and Well-being) and SDG 5 (Gender Equality).

Impact Visual board



Project Highlights

ASRH Study – Wave III

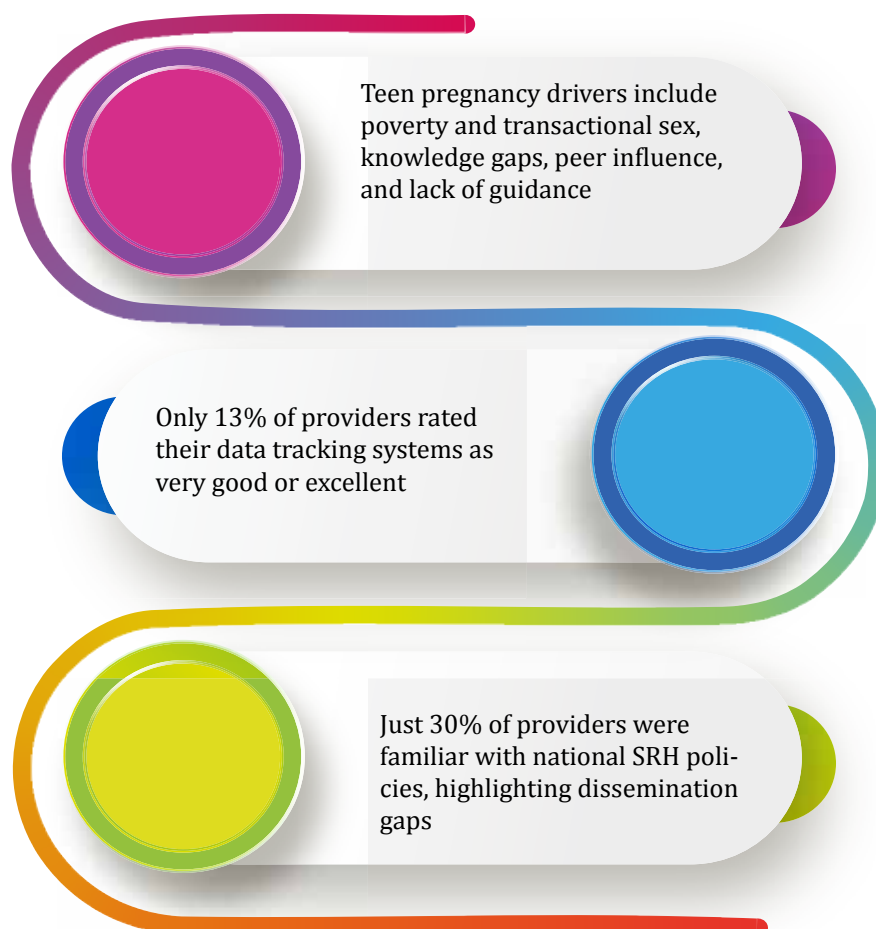
ICRHK implemented the third wave of the Adolescent Sexual and Reproductive Health and Rights (ASRHR) Study to assess the current landscape of adolescent SRH in Kisumu, Trans Nzoia, and Migori Counties. The study examined policy implementation, service provision trends, and stakeholder attitudes to inform county-level programming, planning, and monitoring.

The assessment engaged 62 health facilities, including 30 service providers, 32 Community Health Promoters (CHPs), and 15 County Health Management Team (CHMT) members, strength-

ening local ownership of findings and response planning.

Findings underscored that improving adolescent SRH outcomes requires action beyond facilities, including stronger policy dissemination, governance oversight, commodity availability, provider capacity, and community engagement. County-level dissemination of results strengthened dialogue with leadership, positioning ICRHK as a technical partner in advancing evidence-informed ASRH programming.

Key Findings



Strategic National Engagement & Devolution Leadership



ICRHK participating in the panel during the Devolution Conference in Homabay

ICRHK also strengthened its national influence by representing the health sector on the multi-agency planning committee for Kenya’s Devolution Conference. Contributing to dialogue on re-engineering county governments, ICRHK advanced health equity, inclusion, and climate-responsive development within the broader devolution agenda.

ICRHK’s engagement with the Council of Governors (CoG) deepened through sustained consultation and co-creation. In 2025, ICRHK was seconded to the CoG Resource Mobilization and Partnership Committee, reinforcing its role as a trusted technical partner in shaping policy, strengthening county systems, and mobilizing sustainable financing for health.



ICRHK engagement with the Council of Governors

Monitoring & Evaluation

Our Monitoring, Evaluation, Research & Learning (MERL) Department strengthened ICRHK's institutional impact in 2025 by embedding evidence, accountability, and adaptive learning across all programs.

Through robust monitoring systems, national technical assistance, research leadership, and knowledge dissemination, MERL ensured that data translated into strategic decisions, policy influence, and measurable health outcomes.

Despite disruptions caused by the USAID transition, the department sustained program oversight, supported grant development, strengthened national data systems, and reinforced compliance with regulatory and data protection standards.



Nuru ya Vijana team verifying their program data

IMPACT



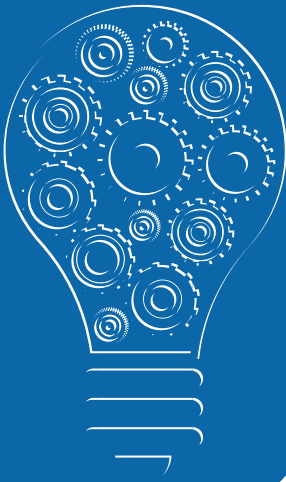
Provided national technical MERL support to Kenya's Ministry of Health across 47 counties



Supported development of 5+ competitive grant proposals and research protocols



Delivered 20+ conference abstracts and 1 peer-reviewed publication (3 manuscripts in pipeline)



Innovation Hub

Our commitment to innovation and epidemic preparedness came to life in 2025 by re-operationalizing the Bionear Mobile Laboratory. Originally a cornerstone of our regional COVID-19 response, the Bionear has evolved into a versatile, high-tech diagnostic hub.

In FY25, it served two critical roles: a shield against emerging infectious diseases and a bridge to life-saving preventive care in underserved regions.



Strengthening Epidemic Preparedness: The Mpox Response

As the Coastal region faced the Mpox outbreak, ICRHK positioned the Bionear as a vital asset for Mombasa County's Department of Public Health.

I M P A C T

By bringing molecular testing (qPCR) directly to the field, we bypassed the logistical delays of centralized laboratory systems.

Bridging the Gap in Ganze: Integrated Community Care

The Bionear Laboratory journeyed to Ganze, transforming a rural health facility into a high-capacity diagnostic center for the day. This deployment proved that the lab's value extends far beyond pandemic response.



Cervical Cancer Screening: 40+ women screened for HPV using GeneXpert, with results available in one hour to support immediate care.



60+ residents received Full Blood Picture (FBP) tests in under 10 minutes, enabling early detection of anemia and infections.



Board Member Lou Dierick showcasing the features of the Bionear Mobile Lab during the 30th AKMLSO Conference in Mombasa

" The Bionear is more than a lab; it is our promise that high-quality healthcare will no longer be limited by geography. Whether it is a global outbreak like Mpox or a routine screening in Ganze, we are bringing the hospital to the people. — **Lou Dierick – Board Member**

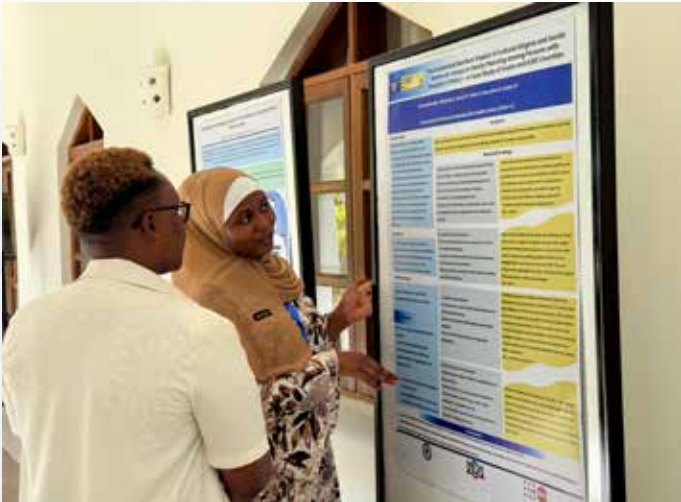


Bionear Laboratory team members during the presentation of Bionear lab at the 30th AKMLSO Conference

Snapshots Corner

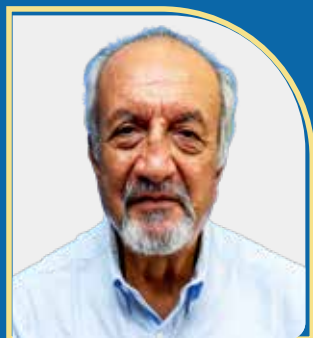






Our Team

Board Members



Prof. Kishor Mandaliya
Chairperson



Prof. Walter Jaoko
Secretary



Dr. Esther Getambu
Treasurer



Prof. Violet Naanyu
Board Member



Mr. Lou Dierick
Board Member



Dr. Jenniffer Othigo
Board Member



Dr. James Odera
Board Member

Senior Management Team



Dr. Susan Ontiri
Country Director/CEO



Dr. Dismas Congo
Technical/Research Lead



Ms. Mary Thiongo
Project Director



Irene Kiminza
Head of Finance/
Administration & Op-
erations

Our Partners

Donors



AGA KHAN FOUNDATION



Government & Policy Partners



MINISTRY OF HEALTH



THE AGA KHAN UNIVERSITY

Our Publications

TITLE OF ABSTRACT	CONFERENCE
“Undetectable=Untransmittable (U=U) Messaging Effect on Viral Suppression, Stigma Reduction, and Community Engagement among HIV-Positive Female Sex workers in Mombasa, Kenya”	15th KASH CONFERENCE 2025
PMA Agile: Best Practices and Lessons Learnt	Kenya Population and Development Conference 2025
Disregarded Abuse: Examining forms of economic abuse among young women in intimate partnerships in Nairobi Kenya	Kenya Population and Development Conference 2025
Intersections of Climate Change, Child Rights, and Environmental Stewardship in Kwale County, Kenya: Finding from Linda Mtoto Child Rights Situational Analysis Study	Kenya Population and Development Conference 2025
Facilitators and Barriers of Adolescent Sexual Reproductive Health in Kenya	Kenya Population and Development Conference 2025
Legal Outcomes of Sexual Violence Court Cases in Mombasa, Kenya	African Workshop on Women & HIV 2025
Adopting a Youth Peer Provider Model for Responsive Adolescent Sexual Reproductive Health	XXV FIGO World Congress of Gynecology and Obstetrics
Understanding the Decision-Making Dynamics to Termination of Pregnancy among Adolescent Girls and Young Women 10 -24 Years in Kenya	30th International Population Conference (IPC2025)
The Overlooked Barriers: Impact of Cultural Stigma and Social Norms on Access to Family Planning among Persons with Disability	Kenya Population and Development Conference 2025
Without them, many children would suffer in silence” Role of Child Protection Volunteers in strengthening child safeguarding systems in Kwale County, Kenya. A qualitative study.	ISPCAN Congress
Addressing HIV and SRH needs among young female sex workers and transgender youth in Kilifi County	University of Nairobi STD/HIV/SRH collaborative research group
Evaluation of depressive symptoms, HIV vulnerability, and oral PrEP uptake and adherence among adolescent girls and young women in Western Kenya: a cross-sectional study	University of Nairobi STD/HIV/SRH collaborative research group
Sex workers champions role in addressing PrEP continuation in Kilifi County – a case study	University of Nairobi STD/HIV/SRH collaborative research group
U=U (Undetectable=Untransmissable) messaging effect on viral suppression, stigma reduction, and community engagement among HIV-positive female sex workers in Mombasa, Kenya	University of Nairobi STD/HIV/SRH collaborative research group
Pathways for Transitioning to a Diagnostic-Led Approach for STI Management in Kenya	STI & HIV World Congress 2025

Stakeholder Perspectives on Adopting Point-of-Care Diagnostics for STI Management in Kenya	STI & HIV World Congress 2025
Intersections of Climate Change, Child Rights, and Environmental Stewardship in Kwale County, Kenya: Finding from Linda Mtoto Child Rights Situational Analysis Study	Kenya Population and Development Conference 2025
Impact of Anti-LGBTQ Campaigns on STI Incidence Among Key Populations in Mombasa, Kenya: A Case Study of ICRHK.	IAS Conference 2025
Addressing Stigma Faced by Young Female Sex Workers and Transgender Individuals in Kilifi County, Kenya via Structural Reforms and Community Advocacy	INTEREST 2025
Enhancing HIV Prevention for Young Key Populations in Resource-Limited Settings through Scalable Interventions in Kilifi County, Kenya	INTEREST 2025
Transforming HIV Outcomes for Marginalized Populations in Kilifi County, Kenya through Peer-Led Outreach and Civil Society Engagement	INTEREST 2025
Bridging the gap: Integrating post-abortion care into pharmacy-based care in Kenya via community and peer health workers	2025 FIGO Conference
The Overlooked Barriers: Impact of Cultural Stigma and Social Norms on Access to Family Planning among Persons with Disabilities (PWDs) – A Case Study of Kwale and Kilifi Counties	Kilifi County Scientific Conference 2025
Exploring Menstrual Health and Hygiene Practices among Adolescents Aged 10–19 Years in Kenya	Kilifi County Scientific Conference 2025
Facilitators and Barriers of Adolescent Sexual Reproductive Health in Kenya	Kilifi County Scientific Conference 2025
Addressing HIV and SRH Needs Among Young Female Sex Workers and Transgender Youth in Kilifi County, Kenya	Kilifi County Scientific Conference 2025
Facilitators and Barriers to Integrating Point-of-Care STI Testing in Kenya: Qualitative Findings	Kilifi County Scientific Conference 2025
Utilization of Post-Abortion Family Planning Services in the Private Sector Among Adolescents and Young Women in Kenya.	ICFP 2025
Insights from Exemplars: Government-driven solutions for advancing family planning and SDGs.	ICFP 2025
Understanding the Decision-Making Dynamics to Termination of Pregnancy among Adolescent Girls and Young Women 10 -24 Years in Kenya	30th International Population Conference (IPC2025)

Facilitating Adolescent Access to ASRHR Services: A Public-Private Referral Network Approach	2025 FIGO Conference
Health system readiness for a diagnostic-led approach to STI management in Kenya: A mixed methods study	2nd Nairobi Health Scientific Conference

Manuscript

Positioning justice: a legal cascade of sexual violence cases in Mombasa, Kenya

<https://www.frontiersin.org/journals/global-womens-health/articles/10.3389/fgwh.2025.1605612/full>

Evaluation of depressive symptoms, HIV vulnerability and oral PrEP use and adherence among adolescent girls and young women in Western Kenya: a cross- sectional study

<https://bmjopen.bmj.com/content/bmjopen/15/10/e100011.full.pdf>

Determinants of health facility choice for delivery among women participating in group antenatal care in Machakos county, kenya: A cross-sectional survey

<https://reproductive-health-journal.biomed-central.com/articles/10.1186/s12978-025-02107-w#citeas>

Intimate partner violence help-seeking norms: scale reliability and cross-sectional multilevel associations with intimate partner violence among youth in Nairobi, Kenya

<https://pubmed.ncbi.nlm.nih.gov/39809554/>

Enhancing accountability in maternal and newborn health referrals through a network of care: A mixed methods implementation research in Makueni, Kenya

<https://www.sciencedirect.com/science/article/pii/S2949856225001072>

Maximizing Group Antenatal Care (G-ANC) coverage in health facilities: Study Protocol for an Implementation Research in Machakos County, Kenya

<https://gatesopenresearch.org/articles/9-34/pdf>





International Centre for Reproductive Health-Kenya

Mombasa Office

ICRHK Tudor Office, Off Nyali Bridge, next to MM & MV Shah
Academy-Tudor Mombasa, Kenya

Nairobi Office

Starwood Apartments 2nd floor B5, Argwings Kodhek Road,
Hurlingham

P.O. Box 91109 – 80103, Mombasa, Kenya

info@icrhk.org

+254 41 249 4866 / +254 722 208 652

www.icrhk.org